

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

MARTINE BOURGOIN

Plaintiff

- and -

THE CORPORATION OF THE
MUNICIPALITY OF LEAMINGTON

Defendant

)
)
) William C. Chapman and Melanie A.
) Gardin, for the Plaintiff
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) Brian McCall, for the Defendant
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)

) **HEARD:** January 3, 4, 5, 6, 9, 10, 11, 12,
) 13, 16, 17, 18, 19, 20 and 23, 2006

Nolan J.

INTRODUCTION

[1] Martine Bourgoin claims damages against the Corporation of the Municipality of Leamington for injuries arising from a slip and fall accident which occurred at about 8:45 p.m. on July 6, 2000. While walking with a friend, Ms. Bourgoin fell on a sidewalk on the west side of Erie Street in Leamington and injured her right ankle. The defendant has denied liability for the accident.

[2] At the time of the fall, Ms. Bourgoin was an active 45-year-old woman, in good health, living with her husband and three of her four daughters, ages 11 to 16, and was working at Highline Produce where she had worked since 1996.

[3] At the time of the trial, Ms. Bourgoin was 51 years old. Her husband had left her in July 2005. She is unemployed and in receipt of public assistance.

[4] Ms. Bourgoin's mother tongue is French and while she speaks and reads English, she does so with a pronounced accent. At various times during the trial she had difficulty understanding some words or phrases in questions she was asked by both her own counsel and counsel for the defence.

[5] At the commencement of the trial I agreed to the request of counsel that the trial be heard in two phases. The issues to be determined at phase one were framed as questions by counsel and included: 1) liability of the municipality; 2) any contributory negligence by Ms. Bourgoin; 3) the nature and extent of her injuries; 4) damages to which Ms. Bourgoin may be entitled and whether she has an obligation to mitigate any damages by having an amputation of her right leg below the knee; 5) special damages for past care; 6) special damages past wage loss; 7) future loss of income; and 8) what, if any, future care Ms. Bourgoin may require, including whether she is more likely than not to have a below the knee amputation at some point in the future. The issue of any loss of shared family income raised by plaintiff's counsel at trial was withdrawn. The answers to the specific questions posed by counsel are found in summary form at the conclusion of this judgment.

1) LIABILITY

The Accident

[6] Erie Street in Leamington runs north and south to Lake Erie. It is a main street that is heavily travelled and a popular walking route to the lake. Ms. Bourgoin was walking in a southerly direction with her friend, Raymonde Colasanti, as was their custom three times per week from approximately June to September. Ms. Bourgoin was wearing walking attire and Dr. Scholl's walking shoes.

[7] The evening was warm and clear. The sidewalk was dry and visibility was good. As Ms. Bourgoin walked and talked with Ms. Colasanti, looking ahead, not down, she fell to the sidewalk unexpectedly and without warning. After the fall she noticed an indentation in the sidewalk. She went over on her ankle and was in pain. She said she felt nauseous and urged Ms. Colasanti to wait a minute before trying to help her up. Ms. Bourgoin said her ankle changed colour and started to swell.

[8] Exhibits 5 and 8 contain photographs of the sidewalk taken on behalf of both Ms. Bourgoin and the municipality. The location of her fall is marked with an "X", and the indentation in the sidewalk is shown to be between 1¼" to 1½" deep at the edge. The photographs also show other areas on the sidewalk that had been repaired previously.

[9] After Ms. Colasanti helped Ms. Bourgoin get up, a man on a bicycle who had passed by them going in the opposite direction moments before the fall, came back. Ms. Colasanti used the man's cell phone to attempt to call Mr. Bourgoin. When that attempt was unsuccessful, Ms. Colasanti helped Ms. Bourgoin hop across the street to a bench where they flagged down a cab. He took them to the Bourgoin home from where Ms. Colasanti took Ms. Bourgoin to Leamington General Hospital. There she was x-rayed, given medication and told to get a tensor

bandage. Ms. Bourgoin went to see her family doctor, Dr. Gorrell, several days later. Reports from the hospital and Dr. Gorrell are found in Exhibit 1. The plaintiff did not get the names of either the bike rider or the cab driver. The only witness to the accident, other than Ms. Bourgoin, was Ms. Colasanti, who testified at trial.

[10] On cross-examination, Ms. Bourgoin acknowledged that not all sidewalks are even and that one had to keep a lookout when walking. She agreed that she was familiar with the area as she had walked the same sidewalk at least three times per week during the summer months for the three years before the accident. She acknowledged that she had walked over the same sidewalk between 8 and 12 times in the weeks before the accident.

[11] Exhibit 8 contained pictures taken on behalf of the municipality. Some of them were taken by Mr. Settington, the insurance adjuster, on or about August 2, 2000. In the picture taken on August 2, 2000, the indentation in the sidewalk was less visible than it was in the picture taken by Mr. Settington on August 11, 2000 when he subsequently attended the scene with Ms. Bourgoin. In the latter picture, the indentation was more visible. The evidence was that it had rained on the morning of August 11, 2000. The result of the moisture was that the indentation looked a different colour than the rest of the sidewalk, including the other parts that had been repaired previously. Ms. Bourgoin testified that on the night of the fall she could not have seen the indentation, even if she had been looking.

[12] The other witness to the fall, Raymonde Colasanti, has known Ms. Bourgoin and her family for 25 years. She and Ms. Bourgoin started walking together in 1997. They would walk from a coffee shop called Blondie's to the lake, take a rest and then walk back, taking the same Erie Street route each time. Ms. Colasanti said Ms. Bourgoin always walked on the side of the sidewalk closest to the road because she, Ms. Colasanti, preferred to walk furthest away from the road.

[13] Ms. Colasanti described the evening as beautiful. She said that they were walking along talking when, all of a sudden, Ms. Bourgoin "dropped". She thought Ms. Bourgoin had fainted. She said Ms. Bourgoin was crying and at first would not let Ms. Colasanti help her up. Ms. Colasanti said she did not see the hole before Ms. Bourgoin fell and that one could not see it because it was shallow and the same colour as the sidewalk.

[14] Ms. Colasanti confirmed that Ms. Bourgoin wore running shoes. She described her foot at the hospital as being three times the normal size. The doctor, who Ms. Colasanti described as not being very patient or sympathetic, told them that Ms. Bourgoin had probably just sprained her ankle. Ms. Colasanti said that when she saw her foot a few months later it was not quite so swollen but that it was very dark blue or black.

[15] On cross-examination, Ms. Colasanti acknowledged that she did not know why Ms. Bourgoin fell. She acknowledged that in the picture taken August 11, 2000, she could see where the indentation was because it was wet but said that it would be hard to see if it was not wet. Ms. Colasanti did not see the hole until after the fall.

[16] Ms. Bourgoin testified that after the accident, she had called her sister, Claudette Lamarr, to tell her about the fall and to ask for her help. Ms. Bourgoin does not think she speaks English well, French being her first language. Ms. Lamarr called the municipality on behalf of Ms. Bourgoin and spoke with a town employee, Mr. Sweet. As a result of that call, Mr. and Ms. Bourgoin went to talk to Mr. Sweet who said he would send an insurance adjuster to see her.

[17] Ms. Lamarr also helped Ms. Bourgoin write a letter to the municipality that was put into evidence as Exhibit 10. In that letter dated July 18, 2000, Ms. Bourgoin described the accident, described her injuries, the medical care she received and the fact that she was off work as a result of the fall. In the letter, Ms. Bourgoin said that her “inability to work has not only put a great strain on my physical being it has also affected our personal finances”. She went on to describe the financial difficulties that the accident had caused. She requested that the municipality pay for all her “health care costs and lost wages be fully restored from the onset of my injury to the date of my full recovery”. Although it was never produced by either Ms. Bourgoin or the municipality, the letter referenced a diagram that was supposedly attached to the letter, showing the location of the fall. The absence of the sketch is not significant since there is no dispute as to where the accident occurred.

Was The Municipality Negligent?

[18] The only person who testified at the trial for the municipality on the issue of liability was Paul Settingington, the insurance adjuster. Richard Leslie, manager of Maintenance and Public Works was examined for discovery as a representative of the municipality. He did not testify at the trial and some of the questions and answers from that discovery were read into the record by the plaintiff’s counsel. I will deal with those later.

[19] Mr. Settingington, a licensed insurance adjuster with 40 years experience, had investigated the fall for the municipality. On or about July 27, 2000, he had been provided with the Exhibit 10 letter, but not the sketch. When he went looking for the location of the fall, he could not find it by walking down Erie Street in a southerly direction. He saw a few areas where he thought someone could fall and he took a picture seen in Exhibit 5 at Tab 2 to show the general condition of the sidewalk and area. On or about August 1, 2000, he contacted Ms. Bourgoin to get more information regarding the actual location. The next day, he took four photos which were produced at Tab 3 of Exhibit 5. The part of the sidewalk where Ms. Bourgoin said she fell was marked with an “X”. This was an indentation in the cement of unknown origin about 1¼” deep at the edge. This depth is indicated in the photo by way of a ruler placed vertically in the indentation. Mr. Settingington looked around the area and returned to the location of the fall on August 11, 2000 with Ms. Bourgoin. She commented at that time that she was surprised the indentation had not yet been repaired. When Ms. Bourgoin and her husband went back to the location of the fall at about 5:00 p.m. on August 11, 2000, the indentation had been patched, as seen in the picture they took at that time.

[20] On cross-examination Mr. Settingington acknowledged that the indentation was deeper than a ½” depression which, according to Mr. Leslie’s discovery testimony, required repair in

accordance with the town's policy. He could not remember if he had called the municipality on August 2, 2000 when he first identified the area. He agreed that while he could see the indentation from three to four feet away, what one sees when investigating a slip and fall can be different from what one sees walking ordinarily.

[21] Counsel for Ms. Bourgoin read a number of questions asked and answers given on the examination for discovery of Richard Leslie into the record. On the examination, he said that the municipality allocated approximately \$20,000 each year for sidewalk repairs but had run out of money before July 6, 2000. The area where Ms. Bourgoin fell had been slated for replacement as shown by the white painted arrow identified in a photograph. The mark had been put there by him to identify the stretch of sidewalk to be replaced.

[22] Mr. Leslie also acknowledged that even if the sidewalk was not reached for replacement, it should have been patched. Mr. Leslie acknowledged that in addition to the \$20,000 allocated to the repair of sidewalks, there was a maintenance schedule in place. All sidewalks are to be inspected twice a year. The maintenance person brings cold patch with him or her. Every section is inspected and any areas that are approximately a half-inch or deeper are cold patched and documented. Mr. Leslie said that the indentation where the fall occurred would have been in that condition for about six months and that the area should have been inspected and patched prior to July 6, 2000. He described how the inspectors ride a tractor, not walk on the sidewalks and that, had the inspectors seen it, it should have been patched. No inspector testified at the trial.

Summary of Factual Findings

[23] Based on the evidence given by Ms. Bourgoin, Ms. Colasanti, Mr. Settingington and the questions and answers read into evidence of Mr. Leslie's discovery, I make the following findings of fact: Ms. Bourgoin fell while she was walking with her friend on Erie Street, a heavily travelled sidewalk on the main road to the lake, looking ahead, not down, when her right foot stepped into or on the edge of the depression in the sidewalk which was in excess of 1¼" deep.

[24] At the place where she fell, the depression exceeded the ½" depth that the municipality had set, as a policy, the level at which a crack or depression in a sidewalk should be repaired.

[25] The sidewalk where Ms. Bourgoin fell had been identified within the preceding months as part of an area to be replaced, but the municipality's budget for sidewalk repair was exhausted less than two months into the repair season.

[26] Based on the answers to the questions on discovery given by the municipality's representative, Mr. Leslie, the area should have been patched, if the workers had actually inspected it and noticed it. Mr. Leslie said that the municipality's maintenance procedure required that all town sidewalks were to be inspected twice a year, and that the worker inspecting the sidewalks would have cold patch asphalt with him or her to immediately do the repair. No inspection records were produced at trial.

[27] The municipality knew or ought to have known that area of the sidewalk posed a hazard to pedestrians. When the budget restrictions prevented the replacement of the sidewalk, knowing the condition of that part of the sidewalk, it should have been patched.

[28] Since that part of the sidewalk was already identified as needing replacement, further inspections would only serve to reinforce the determination that had already been made about the condition of the sidewalk.

[29] The fact that the depression was not readily visible made it more of a hazard. The sidewalk was in a state of non-repair and this state caused the plaintiff's fall and her injured ankle.

The Law

[30] Municipalities are not to be held to a standard of perfection. Whether or not certain acts or omissions constitute negligence depends on the facts of each case. The relevant statute is the *Municipal Act*, R.S.O. 1990, c. M. 45. In particular, s. 284 provides as follows:

284. (1) - The council of the corporation that has jurisdiction over a highway or bridge shall keep it in a state of repair that is reasonable in light of all the circumstances, including the character and location of the highway or bridge.

(1.1) – In case of default, the corporation, subject to the *Negligence Act*, is liable for all damages any person sustains because of the default.

(1.2) – The corporation is not liable under subsection (1) or (1.1) for failing to keep a highway or bridge in a reasonable state of repair if it did not know and could not reasonably have been expected to know about the state of repair of the highway or bridge.

(1.3) – The corporation is not liable under subsection (1) or (1.1) for failing to keep a highway or bridge in a reasonable state of repair if it took reasonable steps to prevent the default from arising.

[31] In an action against a municipality on account of a slip and fall, a plaintiff must first establish, on the balance of probabilities that the place where the fall occurred was in a state of non-repair. Then, the plaintiff must establish a causal link between the state of non-repair and the injury. If the plaintiff satisfies these evidentiary requirements, the municipality must then establish that it has a reasonable system in place for inspection and repair. If the municipality can meet this burden, it will not be held liable. Otherwise, as MacFarland J. (as she then was) held in *Stojadinov v. Hamilton (City)* (1988), 41 M.P.L.R. 185, the municipality would virtually be the insurer of every pedestrian who walked its streets. Further, a pedestrian is not entitled to expect that the sidewalk will be as “smooth as a billiard table”, a principle enunciated in a number of

cases including *Gilmour v. City of Toronto*, [1926] O.J. No. 456, *Cock v. Windsor*, [1944] 2 D.L.R. 778, *Blaquiere v. Burlington (City)*, [1999] O.J. No. 2558 and *MacKay v. Thunder Bay (City)*, [2000] O.J. No. 2953.

[32] The *Municipal Act*, R.S.O. 1990, c. M. 45, s. 248 (1.3), set out above, as well as case law, including *Michaluk v. Oakville (City)*, [2000] O.J. No. 4466 and *Blaquiere v. Burlington (City)*, *supra*, provide that a municipality is not liable for failing to keep a sidewalk in a reasonable state of repair if it took reasonable steps to prevent the default from arising. These two cases also stand for the proposition that it is not unreasonable for a municipality to take financial considerations into account along with physical considerations. The determination of whether non-repair exists and whether the disrepair caused the incident depends on the facts of each individual case.

[33] With respect to determining the extent to which the state of disrepair caused or contributed to the fall, in a very recent decision of the Ontario Court of Appeal, *Kamin v. Kawartha Dairy Limited* (February 8, 2006), the decision of the trial judge, reported at [2004] O.J. No. 1056, was overturned. The plaintiff in that case fell in the parking lot of an ice cream dairy. Although the trial judge had found that the inspection system in place for the parking lot by the dairy was inadequate and failed to meet the standard set out in s. 3(1) of the *Occupier's Liability Act*, R.S.O. 1990, c.O. 2, and further, that the parking lot was in a state of disrepair, the plaintiff's claim was dismissed. Relying on the decision of Urquhart J. in *Cock v. Windsor*, the trial judge held the plaintiff had not met the onus of establishing where she fell, that there was a depression at that location and that the depression was the cause of the fall.

[34] Borins, J.A., writing for himself and Simmons and Armstrong JJ.A., said that the principles articulated in *Cock v. Windsor*, with respect to proof of causation were no longer the law, having been replaced by *Snell v. Farrell* [1990] 2 S.C.R. 311 which held that causation need not be determined by scientific precision, that causation is "essentially a practical question of fact which can best be answered by ordinary common sense rather than by metaphysical theory." Borins J.A. then referred to *Athley v. Leonati* [1996] 3 S.C.R. 458 which applied *Snell v. Farrell*, *supra*, and held that liability is established where the defendant's negligence caused or materially contributed to the plaintiff's injury.

[35] Borins J.A. said that when there is sufficient evidence on which to find that a plaintiff's injuries are caused or materially contributed to by a defendant's negligence and there is no evidence of any other probable reason for a plaintiff to have fallen, it is an error of law to fail to find or to draw the only reasonable inference that the defendant's negligence caused or materially contributed to the plaintiff's fall and her resulting injuries.

Conclusions on Liability

[36] The application of the law to the findings of fact in paragraphs 23 to 29 above, supports a finding of negligence against the municipality. Ms. Bourgoin knew where she fell, there was a depression in the sidewalk of a depth that, according to the policy of the municipality should

have been repaired. The fact that it was a heavily travelled and popular walking route and there was no evidence of others falling there, does not excuse the municipality on the facts of this case.

[37] The municipality had a plan to replace a stretch of sidewalk that included the area where Ms. Bourgoin fell. While it may have been reasonable for the municipality not to replace the sidewalk because of budgeting constraints, it does not excuse its failure to follow its own policy of inspecting and patching areas of sidewalk with depressions over ½ inch in depth. If the municipality knew that a portion of sidewalk was in a condition that warranted replacing and the replacement could not be done for budgetary reasons, there was an obligation on the municipality to pay particular attention on inspection to that area and the need for patching. Erie Street is a popular pedestrian walk to the lake. Based on the documentary evidence, witness testimony, and the municipality's failure to follow and carry out its own policies for inspecting and patching sidewalks, I find the municipality is liable for the state of non-repair of the sidewalk and find the municipality to be negligent, causing Ms. Bourgoin to fall.

2) CONTRIBUTORY NEGLIGENCE

[38] The municipality asserted that if I found it to be negligent, I should find that Ms. Bourgoin was contributorily negligent and find that she should bear at least one third of the responsibility for the fall. The municipality submitted that Ms. Bourgoin should have been alerted to the possibility of the existence of uneven conditions since there were at least three other patched areas near the location of the depression in the sidewalk. As well, she was familiar with the area, having walked over it many times over the years and, in particular, 8 to 12 times in the several weeks preceding the fall.

[39] In support of the principle that pedestrians have a duty to keep a proper lookout, counsel for the municipality referred me to a number of cases. In *Cooney v. Kingston (City)*, [2005] O.J. No. 2196, the plaintiff who slipped on a ridge of ice in the month of February was found to be contributorily negligent and liability was divided 50/50.

[40] In *Belanger v. Michipicoten (Township)*, [1996] O.J. No. 275 the plaintiff fell on a patch of ice in the month of March. The plaintiff was found to be contributorily negligent and the court assessed the plaintiff's negligence at one third.

[41] In *Bognoch v. Toronto (City)*, [1991] O.J. No. 1032, the plaintiff fell on an icy sidewalk also in the month of February. The court attributed her contributory negligence at 40 per cent.

[42] In *Michalak v. Oakville (City)*, [2000] O.J. No. 4466, a plaintiff, walking in an unfamiliar area, tripped on a crack that created a height difference of less than an inch. The day was bright and clear and the crack was readily visible even from across the road. The plaintiff's contributory negligence was apportioned at 25%.

[43] The municipality's counsel urged me to find that Ms. Bourgoin was contributorily negligent. She was familiar with the area. She acknowledged that she had a responsibility to keep a proper lookout and that a pedestrian could not expect a sidewalk to be perfectly smooth. He

submitted that if Ms. Bourgoin had been looking where she was going, she could have avoided the depression in the sidewalk. He urged me to find that she was not watching where she was going and that she did not keep a proper lookout.

[44] It was Ms. Bourgoin's position that she was not contributorily negligent. She was wearing proper footwear as confirmed by Ms. Colasanti. She had not been drinking. She was a frequent and experienced walker. She said, in fact, that walking was one of her favourite activities, and she was not prone to falls. This love of walking was confirmed by the testimony of Ms. Colasanti and the plaintiff's daughter, Natalie.

The Law

[45] The onus is on the municipality to establish on the evidence that Ms. Bourgoin was contributorily negligent. It is not the law that if a pedestrian falls on a sidewalk, and the municipality is found in breach of its statutory duty and the negligence of the municipality is found to have caused the fall, the plaintiff must always be partly responsible for the fall since he or she should have seen the non-repair in the sidewalk and avoided it.

[46] In *Assman v. Etobicoke (City)*, *supra*, the trial judge found that the plaintiff was not contributorily negligent, in spite of falling on the street where she lived, on a part of the sidewalk that she walked over at least twice a day, going and coming from work. Even though finding that the plaintiff was not looking down, as I have also found in the case before me, the trial judge found that there was no obligation to do so. In coming to that decision, Sanderson J. relied on a number of cases.

[47] In *Snitzer v. Becker Milk Co. Ltd.* (1976), 15 O.R. (2d) 345 (H.Ct.), Lerner J. held at p. 355:

The plaintiff was entitled to expect that the sidewalk was in good condition from which he could assume that adjoining slabs of concrete were reasonably level. Pedestrians are not expected nor required to walk with their eyes focused downward immediately in front of their feet to make certain that the slabs of the sidewalk are reasonably level, particularly a sidewalk in front of a row of adjoining stores in a shopping plaza.

[48] In *Thorimbert v. Trial (City)*, [1988] B.C.J. No. 2228, Cooper L.J.S.C. found liability against a municipality in a case where the plaintiff stepped into a depression of one to one-and-a-half inches deep on a sidewalk, lost his balance and fell. At page 10, the trial judge said:

The question remains whether the plaintiff used reasonable care for his own safety when using the sidewalk? The depression in question was, as noted, 1 – 1½" in depth. It consisted of non-compacted earth and, according to the evidence of the plaintiff which I accept, was covered with rainwater thus obscuring its depth.

I am satisfied that the plaintiff used reasonable care in stepping onto the sidewalk. He had no reason to suspect that the City would not cover the depression and thereby create a hazard.

[49] In *Jones et al. v. The City of Vancouver et al.* [1979] 2 W.W.R. 138, Anderson J. said at page 140:

While the depressed area around the manhole was there to be seen if a pedestrian looked down directly at the area surrounding the manhole, I do not think there was any duty on the plaintiff to look downward.

Summary of Findings on Contributory Negligence

[50] I find on the facts of this case that Ms. Bourgoin was walking on the sidewalk, talking with her friend. She was wearing proper shoes. There is no evidence she had been drinking or engaging in any other activity that might interfere with walking properly. She had never fallen before so she was not prone to falling. Unlike the first three cases discussed above (*Cooney, Belanger and Bognoek*), there were no winter weather conditions that would or should have alerted Ms. Bourgoin to pay particular attention to the path ahead. As well, unlike the plaintiff in the fourth case, Ms. Bourgoin was not unfamiliar with the area where she was walking. The court held in *Michalak v. Oakville (City)*, *supra*, that walking in an unfamiliar area gave rise to a duty to pay particular attention to where one is walking.

[51] While the pictures in Exhibits 5 and 8 show the depression and although Mr. Settingington said he could see the depression from a number of feet away once it was pointed out to him, he could not find it the first time he went looking for it. The evidence showed the depression was difficult to see in dry conditions. The plaintiff did not see the depression nor did her friend. This failure to see the depression does not make Ms. Bourgoin contributorily negligent. On the facts as presented, I find that there is insufficient evidence on which to make such a finding. I find that Ms. Bourgoin was using reasonable care for her own safety and was not contributorily negligent.

3) NATURE AND EXTENT OF THE PLAINTIFF'S INJURIES

Introduction

[52] While there is no doubt that Ms. Bourgoin suffered an ankle injury when she fell, the most significant issue in this trial, after liability, is the nature and extent of her injuries. In this regard, the evidence is contradictory as to whether Ms. Bourgoin suffers from Reflex Sympathetic Dystrophy, Complex Regional Pain Syndrome, Chronic Pain Syndrome or some form of chronic intractable pain.

[53] The emergency room doctor who treated Ms. Bourgoin at Leamington General Hospital diagnosed her injury as a sprained right ankle. His record, found in Exhibit 1, commented that

Ms. Bourgoin had “inverted right ankle while walking. Tender and swollen lateral malleolus.” An unsigned report from the hospital, probably made by the nurse in emergency, said that she “stepped into a hole in the sidewalk about 15 minutes ago. Swelling noted at right lateral ankle. Barely able to stand at the time of the accident.”

[54] Since leaving emergency on July 6, 2000, Ms. Bourgoin has been treated by her family doctor on 39 occasions, by an anesthetist on three occasions, by two orthopedic surgeons a total of eight times, by a physiatrist and doctor of rehabilitative medicine who is also an expert in pain management three times, a neurologist once and a psychologist twice.

[55] In addition, for the purposes of obtaining independent medical reports for the plaintiff, Ms. Bourgoin was seen by a psychiatrist and pain specialist and two orthopedic surgeons.

[56] As well, Ms. Bourgoin was examined by an orthopedic surgeon and a psychiatrist for the purpose of obtaining independent medical reports for the defendant. In total, she has been treated or examined by physicians, including a psychiatrist, on 61 occasions since July 6, 2000 and on two occasions by a psychologist. She has also had physiotherapy and assessments/examinations by non-medical experts, such as rehabilitation counsellors.

Expert Evidence

[57] As with most actions involving personal injuries, the evidence of the medical experts is crucial in assisting the court to understand the nature and extent of the injuries alleged by the plaintiff. In this case, each of the treating and examining physicians and the psychologist filed reports pursuant to the provisions of the *Evidence Act*. In addition, on consent, I had the benefit of hearing oral evidence from Dr. Gorrell, Dr. Merskey and Dr. Deathe on behalf of Ms. Bourgoin and Dr. Soric and Dr. Yovanovich on behalf of the municipality.

[58] Not unexpectedly, the experts did not all agree on the nature or extent of Ms. Bourgoin’s injuries, on the treatment she should receive or whether she suffers from Reflex Sympathetic Dystrophy or Complex Regional Pain Syndrome or some other form of intractable pain. In considering the various opinions and the weight to be given to those opinions, I considered the qualifications of the doctors as described in their testimony and their *curriculum vitae* filed as exhibits. I also considered their experience and knowledge as it related to the issues in this case as well as their familiarity with the facts, and whether his or her testimony was fair. As well, I considered whether any of the experts went beyond impartiality and appeared to become an advocate in attempting to support one side or the other.

[59] Before examining their evidence, I wish to comment briefly on my impression of the five medical experts who testified as well as filed reports regarding Ms. Bourgoin’s current condition and the origins of that condition.

Dr. James Gorrell

[60] Dr. Gorrell has been Ms. Bourgoin's family doctor since approximately 1978. Although he does not have all her records since 1978 because of changing offices and a fire, he confirmed in his testimony the length of time Ms. Bourgoin had been his patient and he produced all his records of the plaintiff from 1995. He also confirmed that her medical history prior to the accident was unremarkable.

[61] Dr. Gorrell through his records and oral evidence provided information about Ms. Bourgoin's current condition and the origins of that condition. From July 8, 2000, two days after the fall to the trial, Dr. Gorrell has seen Ms. Bourgoin on 39 occasions, the last one being December 31, 2005. In many ways, he has the broadest and most complete view of the deterioration of her condition. He made the referral to the first treating orthopedic surgeon, Dr. Pepin. While continuing to treat her, he has received reports and assessments from other treating physicians and was in direct contact with a number of them, thereby remaining abreast of the developments in Ms. Bourgoin's condition and the views and opinions of specialists.

[62] Dr. Gorrell impressed me as being a kind and caring physician as well as being conscientious. He has provided medical care to Ms. Bourgoin in regard to her injury since the accident and has continued to search for medical solutions for her. In addition, he is charge of her medications including the narcotic medication. Dr. Gorrell is not a specialist, however, and where his opinion on chronic pain differs from those expressed by the experts who were called on behalf of both parties, I must give more weight to the latter.

Dr. Merskey

[63] Dr. Merskey is a psychiatrist, an expert in the area of chronic pain and research into chronic pain. His research has been published in leading medical journals, and peer reviewed publications. He has authored and co-authored numerous abstracts and articles on Reflex Sympathetic Dystrophy and Complex Regional Pain Syndrome. His evidence was helpful to me in understanding these conditions and pain in general. As the only psychiatrist who has examined Ms. Bourgoin, his opinion that Ms. Bourgoin's pain is real, not exaggerated and does not have psychiatric origins was given significant weight in the absence of opposing psychiatric views.

Dr. Barry Deathe

[64] Dr. Barry Deathe is a physiatrist and doctor of rehabilitative medicine. He has extensive experience in the area of pain and has authored and co authored an impressive list of articles and other writings. He had seen Ms. Bourgoin on three occasions over a number of months. His credentials and experience are extensive and I found him to be an impressive witness. While his opinion with respect to the advisability of and the likely need for Ms. Bourgoin to have a below the knee amputation was not supported by any other expert, his experience with amputations is significant and his opinion on that issue had to be seriously considered. Dr. Deathe was also an impressive witness because his evidence did not strike me as being partisan. Although in this case he was called as an expert witness on behalf of a plaintiff, he testified, unchallenged, that he is called just as frequently as an expert witness on behalf of defendants. He answered questions

put to him on cross-examination in an open, fair and straightforward manner. Dr. Deathe had no reservations about Ms. Bourgoin's truthfulness or sincerity when describing her symptoms.

Dr. R. Soric

[65] Dr. R. Soric, a physiatrist and doctor of rehabilitative medicine, was called on behalf of the municipality. She had seen Ms. Bourgoin on one occasion in July 2005. While she had taken a history and observed the appearance of Ms. Bourgoin's feet and ankles, she was unable to physically examine Ms. Bourgoin because of the plaintiff's reaction to pain. Dr. Soric's report described Ms. Bourgoin as presenting "in a most dysfunctional manner." Unlike the other doctors who testified, Dr. Soric's evidence gave a distinct impression of advocacy as she spared with counsel on cross-examination. Her conclusions that Ms. Bourgoin exhibited an "extreme degree of pain focused behaviour," that she was "so exaggerating," as well as concluding that Ms. Bourgoin had no desire to become physically active, were not consistent with the evidence of other medical experts who had treated or examined her.

[66] A troublesome aspect of Dr. Soric's testimony was her acknowledgment that in her initial report of July 19, 2005 had said that Ms. Bourgoin used her *left* foot to push herself up on the examining table. When questioned about that observation in October 2005 by defence counsel, Dr. Soric then changed her report to read that Ms. Bourgoin had pushed herself with her *right* foot when getting up on the examining table, indicating that the word "left" was a typo. This change was made in spite of Dr. Soric's acknowledgment that she had destroyed her clinical notes right after she dictated her report and, therefore, was relying only on her memory several months later. Given the number of patients that Dr. Soric sees in the busy practice she described in her testimony, it calls into question the reliability of her testimony on that point. As well, the surveillance tapes introduced into evidence by the municipality showed Ms. Bourgoin almost always using her left foot to bear her weight when stepping on and off curbs and going up and down stairs.

[67] Dr. Soric also said she had not reviewed the documentation she had received about Ms. Bourgoin prior to meeting with her. Her explanation was that she did not want to "come to the table with pre-conceived ideas." When asked, however, by counsel for the municipality if, after seeing Ms. Bourgoin and forming an impression, "you look for information in the medical documentation as to the – your own diagnosis, you look for information in the medical documentation to support or refute your actual diagnosis", her response was ambiguous, "I actually look at the documentation period. I'm not sure that – yeah, I guess it should be – it is a correct statement." For all those reasons, where her evidence differs from that of Dr. Deathe, I prefer the evidence of Dr. Deathe whose evidence was more consistent with the evidence of other professionals who had examined Ms. Bourgoin.

Dr. Robert Yovanovich

[68] Dr. Robert Yovanovich is an orthopedic surgeon who examined Ms. Bourgoin on February 11, 2003 at request of the municipality. He has written a number of reports, all filed as

part of Exhibit 7. He has impressive credentials in the field of orthopedic medicine and was an effective witness. He did not appear to be partisan in his approach and answered questions put to him on both direct and on cross-examination in an open and professional way. Where Dr. Yovanovich's opinion on the subject of pain and pain syndromes differs from the evidence of Dr. Deathe, however, I accept the evidence of Dr. Deathe since he has a recognized expertise in that area.

The Plaintiff's Health Before Accident

[69] Ms. Bourgoin described her health as being very good and this was confirmed by Dr. Gorrell. She had broken her toe approximately 20 years prior to the fall but the toe had healed and she had no further problems with her foot. She had never injured her ankle before. She had pulled a muscle in her back at work in approximately 1999 but it never resulted in her missing work. She was put on light duties for a couple of days, had an x-ray, was given some medication and it never bothered her again. In 1998 or 1999, Ms. Bourgoin had problems with her right knee. She went to the hospital, had an x-ray, was given medication and her knee got better. Prior to the fall she only went to see Dr. Gorrell, her family doctor, infrequently for such things as the flu, hemorrhoids and the like. This was confirmed by Dr. Gorrell.

Plaintiff's Health After the Accident

[70] The records of Dr. Gorrell show deterioration in the physical condition of Ms. Bourgoin after the fall, including increasing pain, leading Dr. Gorrell to first suspect that she might have Reflex Sympathetic Dystrophy. Dr. Gorrell's records and the drug summaries also reveal regular prescriptions for increased strength drug therapy, including narcotics.

[71] Ms. Bourgoin herself described her increasing inability to walk, to stand for any length of time, burning sensations in her right foot, inability to sleep at night because of pain or to even cover her foot with a light sheet because it would cause pain. Ms. Bourgoin's reaction to the pain was confirmed by her daughters, her sister, her neighbour and her friend, Raymonde Colasanti, all of whom testified. Her difficulty in carrying out her job responsibilities was confirmed by the testimony of Gustavo Goyez and Lorenza Ortiz, both managers at Highline Mushrooms and former supervisors of Ms. Bourgoin as well as the employment records filed by counsel for Ms. Bourgoin. As mentioned above, however, it is the evidence of the medical experts who testified and/or filed reports that carries the most weight in determining the nature and extent of Ms. Bourgoin's injuries, including whether she suffers from a pain syndrome, as a result of the fall.

The Evidence

[72] It is the plaintiff, Ms. Bourgoin, who bears the onus to establish, by way of evidence, the causal connection, on a balance of probabilities, between her present condition and the ankle injury suffered in the fall.

[73] Counsel for Ms. Bourgoin asserted the position that, as in many personal injury cases, other complaints develop following the initial injury. In this case, the principle area of complaint

began with the right ankle, but then extended to the right lower leg and then the lower back. Although no radiology reports prior to July 2000 showed such a condition, further back x-rays done on February 11, 2003 led Dr. Yovanovich to think that Ms. Bourgoin has a defect in her back called spondyloisthesis, which had been asymptomatic prior to the fall. It was suggested in his reports that the low back pain could have been the result of that condition. In his testimony at the trial, however, Dr. Yovanovich said that more likely than not the low back became symptomatic because of Ms. Bourgoin's altered gait and that, but for the injury to her ankle, it was more likely than not that she would have attained 65 years of age without experiencing any low back pain on account of spondyloisthesis.

[74] Dr. Yovanovich disagreed with Doctors Deathe and Merskey that Ms. Bourgoin has Reflex Sympathetic Dystrophy or Complex Regional Pain Syndrome although, unlike Dr. Soric, he did not doubt that her pain was real nor did he think that she was exaggerating her pain. In spite of disagreeing with the diagnosis made by Dr. Deathe, Dr. Yovanovich was unable to definitively diagnose the cause of Ms. Bourgoin's pain. He presented a number of differential diagnoses, none of which were related to any chronic pain syndrome.

[75] At the heart of the differences in the diagnosis of Ms. Bourgoin from the perspective of her experts and those of the municipality, was the absence of consistent observation of changes in her right foot and ankle. Although Dr. Gorrell indicated that he had seen a darkening of the skin of her right foot and had felt it being colder than her left foot, it was the view of both Dr. Yovanovich and Dr. Soric that such symptoms need to be observed at all examinations and that symptoms do not come and go. It was their view that the symptoms reported by Ms. Bourgoin, as well as the observation of her family members and friends are not sufficient to be able to diagnose her problem as Reflex Sympathetic Dystrophy or Complex Regional Pain Syndrome. It was the view of the defendant's experts that the subjective complaints could not be confirmed by observable changes in her foot. The diagnostic tests did not indicate any physiological defect or reason for the complaint of ongoing pain.

[76] With respect to whether Ms. Bourgoin has Reflex Sympathetic Dystrophy or Complex Regional Pain Syndrome, both Dr. Soric and Dr. Yovanovich indicated that these are developmental conditions with specific stages which are easily definable. They both said that the conditions, if they exist, begin soon after an injury and if the condition exists, Ms. Bourgoin would be in the last stages. Trophic changes would be evident and the signs on the examinations would not come and go. They would be seen consistently.

[77] It was the position of the municipality that the medical evidence supports a finding that Ms. Bourgoin has a physiologically unexplained injury with reports of pain out of keeping with physiological findings. As well, the pain as described by Ms. Bourgoin must not be as severe as she indicates it is and the limitations that she says are caused by the injury must not be as significant as she states because there is an absence of muscle wasting. The municipality urged me to find that Ms. Bourgoin's fall and the resulting ankle injury are not causally connected to her current condition.

[78] The evidence that supports the conclusion of Dr. Deathe that Ms. Bourgoin has Complex Regional Pain Syndrome is found in the observations and opinions of other medical experts. In July, 2002, two years after her fall and the pain having spread to her lower leg and lower back, Dr. Gorrell questioned whether Ms. Bourgoin might have Reflex Sympathetic Dystrophy and sent her back to Dr. Pepin, an orthopaedic surgeon who has seen her previously in 2000 and 2001. Dr. Pepin's report of September 26, 2002, commented that her ankle was slightly swollen and that she had a little thickening of the skin. He said, "it is my gut feeling that she has some localized sympathetic dystrophy." Scans are highly inaccurate for making a diagnosis of Reflex Sympathetic Dystrophy.

[79] Dr. Vergel de Dios, the anesthesiologist who administered a series of three sympathetic blocks and questioned whether Ms. Bourgoin had Complex Regional Pain Syndrome. Again in March, 2003, Dr. Pepin thought that Reflex Sympathetic Dystrophy might be a diagnosis.

[80] Dr. Fleming, an orthopaedic surgeon to whom Ms. Bourgoin was referred by Dr. Pepin, said that she had a pain syndrome with what would appear to be a sympathetic dystrophy. He went on to say "her symptoms were suspicious for sympathetic dystrophy with a burning pain which is classically seen with neuralgic or neurogenic pain. I would point out, however, that neurogenic pain is indeed a form of chronic pain and therefore may not be sympathetic dystrophy but simply neurogenic pain."

[81] Dr. Turnbull, another orthopaedic surgeon who saw Ms. Bourgoin for an independent medical opinion thought that she did not have classic sympathetic dystrophy but rather "more of a complex chronic regional pain syndrome for which no one treatment modality will give her complete relief of her pain syndrome." Dr. Turnbull also commented that he did not see any conflict between the various examiners' opinions as to whether or not a Reflex Sympathetic Dystrophy was present, recognizing that even in the absence of any sympathetic dystrophy, patients can have varying degrees of swelling or discoloration often week to week. He concluded that "there is certainly no doubt as to the fact this injury she sustained did cause the subsequent disability and chronic pain that she is experiencing."

[82] Dr. MacLeod, an orthopaedic surgeon who did an independent examination on behalf of Ms. Bourgoin, saw Ms. Bourgoin in October, 2004, did not think that she had Reflex Sympathetic Dystrophy but thought that "chronic regional pain syndrome" would describe his clinical observations and the history of the condition. While he also questioned whether she might have a central lesion, he thought that was quite unlikely.

[83] In April, 2005, Dr. Merskey, a psychiatrist was of the view that Mr. Bourgoin had satisfied all the criteria for a diagnosis of Complex Regional Pain Syndrome as set out at paragraph 87 below. He also disagreed with the opinion of Dr. Soric that Ms. Bourgoin's "pain focused behaviour" was the result of psychological factors. It was the view of Dr. Merskey that the depression and anxiety was the result of the pain.

[84] Throughout all of his contact with Ms. Bourgoin after July, 2002, Dr. Gorrell continued to question whether Ms. Bourgoin had a form of chronic pain syndrome and continued to attempt to find a medicinal solution to the pain, including prescribing Gabapentin, an anti-convulsant which can be effective in neurogenic pain and a narcotic patch.

[85] Dr. MacDonald, a psychologist to whom Ms. Bourgoin was referred by Dr. Deathe for assistance in coping with pain. Dr. MacDonald said that she suffers from chronic pain accompanied by a major depressive disorder with anxiety.

Conclusion on Causation

[86] I find on a balance of probabilities that Ms. Bourgoin suffers from a form of chronic intractable pain resulting from the fall and the ankle injury she suffered. Although there was various views about the condition of her ankle and the likely origins of the pain, there is no other reasonable explanation for her condition other than the fall and the resulting ankle injury. I accept the evidence of Dr. Deathe that Ms. Bourgoin has a form a Complex Regional Pain Syndrome. In coming to this conclusion, I considered all of the medical evidence that was presented by both parties. Some evidence was given more weight, however, because of the experience and qualifications of the doctor who tendered the opinion as well as the number of times the doctors had seen Ms. Bourgoin and the factual basis on which they developed their opinions.

[87] In concluding that Ms. Bourgoin suffers from chronic intractable pain, more likely than not Complex Regional Pain Syndrome, as the result of the injury to her ankle from the fall, I considered the whole of the medical evidence, including the definition of Complex Regional Pain Syndrome and its proposed modified clinical diagnosis. This material was set out in the appendix to Dr. Merskey's report and was referred to by all of the medical experts who testified. The definition and criteria was developed by Doctors Wilson, Stanton-Hicks and Harden who are acknowledged as the leading experts in the area.

General Definition of the Syndrome

CRPS describes an array of painful conditions that are characterized by a continuing (spontaneous and/or evoked) regional pain that is seemingly disproportionate in time or degree to the usual course of any known trauma or other lesion. The pain is regional (not in a specific nerve territory or dermatome) and usually has a distal predominance of abnormal sensory, motor, sudomotor, vasomotor, and/or trophic findings. The syndrome shows variable progression over time.

There are two versions of the proposed diagnostic criteria: a clinical version meant to maximize diagnostic sensitivity with adequate specificity, and a research version meant to more equally balance optimal sensitivity and specificity.

Proposed Modified Clinical Diagnostic Criteria for CRPS

- 1) Continuing pain, which is disproportionate to any inciting event.
- 2) Must report at least one symptom in *three of the four* following categories:

Sensory: Reports of hyperesthesia and/or allodynia

Vasomotor: Reports of temperature asymmetry and/or skin color changes and/or skin color asymmetry.

Sudomotor/Edema: Reports of edema and/or sweating changes and/or sweating asymmetry.

Motor/Trophic: Reports of decreased range of motion and/or motor dysfunction (weakness, tremor, dystonia) and/or trophic changes (hair, nails, skin).

- 3) Must display at least one sign* at time of evaluation in *two or more* of the following categories:

Sensory: Evidence of hyperalgesia (to pinprick) and/or allodynia (to light touch and/or deep somatic pressure and/or joint movement).

Vasomotor: Evidence of temperature asymmetry and/or skin color changes and/or asymmetry.

Sudomotor/Edema: Evidence of edema and/or sweating changes and/or sweating asymmetry.

Motor/Trophic: Evidence of decreased range of motion and/or motor dysfunction (weakness, tremor, dystonia) and/or trophic changes (hair, nails, skin).

- 4) There is no other diagnosis that better explains the signs and symptoms.

[88] The majority of the experts who testified agreed that the former diagnostic title for the syndrome, Reflex Sympathetic Dystrophy, was changed to Complex Regional Pain Syndrome as the former name restricted the condition to those cases which involved the sympathetic system. The current definition of Complex Regional Pain Syndrome is more expansive. It refers to pain that is out of proportion to the original injury, if there was an injury, and continues to get worse rather than better.

[89] The main dispute between the experts who testified for Ms. Bourgoin and those who testified for the municipality with respect to the diagnosis was that the criteria which Wilson, Harden-Hicks and Stanton said is required to be observed must, according to the defendant's experts, be observed at each assessment. I accept the evidence of the plaintiff's experts that it is necessary that there be at least one observation of the required indicators but not at all assessments.

[90] This criteria on which the condition is diagnosed is based on subjective and objective reports and observations. Except for trophic changes, the evidence supports a finding that each of the required criteria have been reported or observed.

[91] With respect to the definitive stages of the syndrome that Dr. Soric and Dr. Yovanovich said occur in every case in which Complex Regional Pain Syndrome can be properly diagnosed, the articles submitted by the municipality as exhibit six, *Chronic Regional Pain and Chronic Pain Syndrome* by J.M.S. Pearce, acknowledged at page two of the article:

In most cases CRPS is claimed to have three stages. Often, however, CRPS does not follow this progression. A minority remit spontaneously, some people go into later stages almost immediately. Others remain in Stage 1 indefinitely. No explanation is apparent in the literature for this inconsistent clinical course.

[92] Except for Dr. Soric and possibly Dr. Yovanovich, no other doctor thought that she was magnifying her symptoms in order to gain sympathy or reward. There is no question that, prior to her fall, everyone described Ms. Bourgoin as being a strong person, physically and emotionally. Nothing in her past suggests that she was unable to manage significant challenges and tragedies in her life. Dr. Merskey and Dr. MacDonald are in agreement that her pain is not psychologically based nor does she have any psychopathology that would cause her to fabricate or exaggerate her pain.

[93] With respect to causation, the governing authority is *Athey v. Leonati, supra*. A defendant will be held responsible if its negligence either caused the plaintiff's injury or is a material contributing factor. I have already found that the municipality's negligence caused Ms. Bourgoin's fall that resulted in the injury to her right ankle. On the basis of all of the medical evidence that it is more likely than not that Ms. Bourgoin suffers from chronic intractable pain, more likely than not Complex Regional Pain Syndrome and that on the balance of probabilities, it is the result of the injury to her ankle. The pain has spread to her leg and as a result of her altered gait, the pain has spread to her lower back. Even if Ms. Bourgoin had a pre-existing condition in her back prior to the fall, a defendant, if negligent, is responsible to the plaintiff as he finds him or her. Dr. Yovanovich acknowledged that Ms. Bourgoin more likely than not would have continued to be asymptomatic but for the fall.

As Major J. said in *Athey v. Leonati, supra*,

The causation test is not to be applied too rigidly. Causation need not be determined by scientific precision ...

It is not necessary, nor has it ever been, for the plaintiff to establish that the defendant's negligence was the sole cause of the injury. There will frequently be a myriad of other background events which were necessary pre conditions to be injury occurring As long as the defendant is part of the cause of an injury, the defendant is liable, even though his act alone was not enough to create the injury. There is no basis for a reduction of liability because of the existence of other pre conditions: defendants remain liable for all injuries caused or contributed to by their negligence.

4) NON-PECUNIARY GENERAL DAMAGES

Mitigation of Damages

[94] Having found that Ms. Bourgoin suffers from chronic intractable pain, which Dr. Deathe said is Complex Regional Pain Syndrome, I must determine the quantum of damages to which she is entitled. The municipality, however, has asserted that any damages should be reduced on account of Ms. Bourgoin's failure to mitigate her losses by refusing to undergo the treatment recommended by her physiatrist, Dr. Deathe. Having raised the issue of mitigation, the onus is on the municipality to establish that Ms. Bourgoin could have and should have mitigated her losses by undergoing the recommended treatment. The onus is met where the medical evidence establishes that a plaintiff's refusal of treatment is unreasonable and not in her best interests.

The Law

[95] Both counsel agree that the leading case on the obligation of a plaintiff to mitigate his or her damages by undergoing a reasonable course of medical treatment is *Janiak v. Ippolito*, [1985] 1 S.C.R. 146. In that case, an injured plaintiff refused to follow her doctor's recommendation for back surgery because of a fear of surgery, even though the surgery was given a 70% chance of success and, if successful, the plaintiff had a 100% chance of recovery and the possibility of returning to employment. The trial judge reduced the plaintiff's damages by 30% on account of her refusal to have the surgery.

[96] The principle underlying the duty to mitigate is that a plaintiff is not entitled to be compensated for losses that could have been avoided by taking reasonable steps, including

undergoing surgery or other recommended medical treatment. In other words, where a plaintiff unreasonably and arbitrarily refuses to undergo surgery or other medical treatment, the defendant cannot be required to bear the cost of this refusal.

[97] The determination of whether a plaintiff's refusal of treatment is reasonable is an objective test based on such factors as the degree of risk to the plaintiff from the surgery or other recommended treatment, the gravity of the consequences of refusing it and the potential benefits to be obtained from having it.

[98] The law is also clear that a psychological problem that develops after an injury, as opposed to a pre-existing psychological condition that renders a plaintiff incapable of making a decision with respect to medical treatment, is not sufficient reason to excuse the plaintiff from having the recommended treatment. The test for determining whether a plaintiff's refusal of surgery or other treatment is reasonable is objective. The plaintiff must bear the cost of any unreasonable decision.

[99] The law as articulated in *Janiak v. Ippolito*, *supra* and *Cochrane v. O'Brien*, [2002] N.J. No. 363 makes it clear that if a court finds that a plaintiff is unreasonably refusing to undergo the recommended surgery or treatment, any damages need to be discounted to reflect the chance that the treatment would have been successful. He or she is only entitled to recover those losses that were unavoidable even if the treatment was performed. Further, if a court finds that the refusal of treatment is unreasonable the court must determine what damages are avoidable by assuming that the plaintiff has agreed to the recommended treatment and then take into account any substantial possibility of failure. The amount by which the damages are discounted represents the avoidable loss. Even if a court cannot determine the mathematical odds for the likelihood of surgical success, it does not prevent a finding that there was a failure to mitigate. The court must estimate the value of the chance that the treatment would have reduced or eliminated the plaintiff's loss.

[100] The determination of the reasonableness of the refusal of a plaintiff to undergo recommended surgery or other treatment must also take into account whether there is conflicting expert opinion on the advisability of receiving a particular course of treatment as reasonable, again to be determined on an objective basis. In *Janiak v. Ippolito*, the court dealt with this issue by reference to a number of English cases. The court concluded that as long as a plaintiff follows any one of several courses of treatment recommended by the medical advisors, he or she should not be said to have acted unreasonably, taking into account the degree of risk to the plaintiff from the surgery or other recommended treatment, the gravity of the consequences of refusing it and the potential benefit to the plaintiff by having it.

[101] In *Engel v. Kam-Ppelle Holdings* (1993), 99 D.L.R. (4th) 401 the Supreme Court of Canada restored the decision of the trial judge who found that the plaintiff had not failed to mitigate his damages by deciding not to undergo further testing or procedures. Information from various physicians indicated that they were unsure whether her pain was muscular in nature or caused by a disk protrusion, a condition that could be corrected by surgery. At trial, two medical

experts testified that although a C.T. scan or myelogram might show the exact nature of the injury, back surgery was not recommended unless her pain became unbearable. The plaintiff elected not to undergo further testing or surgery.

[102] In that case, the Supreme Court of Canada also commented on the right of a person to refuse medical testing and the inviolability of the human body. The court said:

The inviolability of the human body is a fundamental legal principle. As stated by this court in *Re Eve* (1986), 31 D.L.R. (4th) 1: ‘Since, barring emergency situations, a surgical procedure without consent ordinarily constitutes battery, it will be obvious that the onus for proving the need for the procedure is on those who seek to have it performed.’ The same principle can be transposed to medical testing. Clearly, the appellant had the right to refuse to undergo medical testing, with a view to determining the exact nature of her injuries.

[103] The Supreme Court of Canada then went on to consider whether the refusal was arbitrary and unreasonable, giving rise to a finding that the plaintiff failed to mitigate her losses, as the Saskatchewan Court of Appeal had found in overturning the trial judge’s decision. The Court of Appeal criticized the finding of the trial judge in this way:

His summary of the evidence may be correct, but it appears to erroneously assume that if the doctors can accept her decision to refuse diagnostic testing and prospective medical treatment, the law should likewise do so.

[104] The Supreme Court of Canada, however, said:

The position that the law should take is highly dependant on the position taken by the medical profession. As stated by Wilson J. in *Janiak*, at p. 14 “it would appear from the authorities that as long as a plaintiff follows any one of several courses of treatment recommended by the medical advisors he consults he should not be said to have acted unreasonably.”

The Supreme Court then went on to discuss the other parts of the objective test of reasonableness in *Janiak*, the degree of risk to the plaintiff and the gravity of the consequences of refusing it and the potential benefits to be derived from it. The court agreed that the same principles applied to tests.

In restoring the trial judge’s decision, the Supreme Court commented:

However, no evidence concerning lack of risk involved in undergoing the test was presented. The gravity of the consequences of refusing the test was not established, as even the respondent’s own expert

suggested that the level of pain suffered by the appellant was such that surgery was not warranted.

Evidence was presented on the issue of the potential benefit that might be derived from surgery. However, given the recommendation that the appellant not undergo surgery unless her condition worsened, the trial judge rightfully gave no weight to this consideration.

[105] In *Silvanuk v. Stevens*, [1999] A.J. 713 the Alberta Court of Appeal held that the trial judge erred when he reduced the damages of a plaintiff injured in a car accident based on findings related to her duty to mitigate and the permanency of her injuries. The Court of Appeal said:

The question of whether a plaintiff has acted reasonably is one of fact. Care must be taken however not to employ the duty to mitigate as a means to artificially reduce the obligation to pay for damages caused by the accident. The question is always whether the plaintiff's conduct was unreasonable, having regard to all the facts and circumstances surrounding that accident.

Application of the Law

[106] In the case before this court, the surgery that is recommended is the below the knee amputation of Ms. Bourgoin's right leg. None of the cases to which I was referred by either counsel dealt with circumstances in which a court has had to make a determination of mitigation by amputation, that is, whether the surgical removal of a limb or part of a limb falls within the range of medical treatment considered to be reasonable.

[107] The municipality argued that since Dr. Deathe recommended that Ms. Bourgoin have her right leg amputated below the knee because it was his opinion that this surgery would have a 95% chance of eliminating her pain, it was unreasonable for her to refuse the treatment. Even though Dr. Deathe acknowledged that the plaintiff was not ready to make the decision to have part of her leg removed, the municipality argued that a psychological problem that prevents Ms. Bourgoin from making the decision is not a relevant consideration in determining the reasonableness of her decision. In taking that position, the municipality relied on *Janiak v. Ippolito* and other cases that followed *Janiak* and asserted that, objectively, the recommendation is reasonable. Dr. Mersky, a psychiatrist and Dr. MacDonald, a psychologist, have said that Ms. Bourgoin does not have any pre-existing psychopathology that renders her incapable of making the decision. Further, the municipality is of the view that the financial consequences of refusing the surgery should not be visited on it. As a result, it asserts that any damages that I award should take into account Dr. Deathe's opinion that there is a better than a 95% probability that an amputation will relieve her pain, and restore function to the extent that she will be able to return to work.

[108] I find as a fact that Ms. Bourgoin has complied with the testing and treatment recommendations and directions of her treating physicians, with a single exception which I will deal with in greater detail later in this section. Dr. Pepin, who was her initial treating orthopaedic surgeon, had no treatment suggestions. Dr. Yovanovich said, in his report of February 11, 2003, that Ms. Bourgoin has had more than adequate treatment, none of which had been particularly helpful. Dr. Fleming, in his report of April 30, 2003, made suggestions regarding various medications, all of which Ms. Bourgoin has tried unsuccessfully or else is currently taking, including narcotic medication. Dr. Fleming also said that surgery, including amputation is not effective in dealing with chronic pain. Dr. Turnbull, who conducted an independent medical for Ms. Bourgoin, commented in his report of January 4, 2004, that she had received all appropriate treatment for chronic pain and had nothing more to suggest. Dr. MacLeod, who was an examining orthopaedic surgeon, after questioning whether Ms. Bourgoin could have a central lesion in the spinal cord, went on to say that it was highly unlikely. Dr. Yovanovich, in his report of March 21, 2005, said a lumber sympathectomy would be inappropriate.

[109] Dr. Merskey and Dr. Deathe thought that a center such as the Cleveland Clinic might assist with other possible diagnosis and also for treatment of Complex Regional Pain Syndrome. They both thought that Dr. Moulin might have some suggestions to treat neuropathic pain. Dr. Deathe did not think an implanted dorsal stimulator would be helpful. Dr. Soric did not think Ms. Bourgoin required any further treatment and said she had already had a “million dollar work-up.” Dr. MacDonald said that Ms. Bourgoin needed rehabilitative psychological treatment to help her cope with her pain, not psychological treatment to cure her pain, as her pain does not have a psychological source. Her anxiety and depression is a result of her pain. He recommended a minimum of 50 sessions. Ms. Bourgoin is planning to continue her treatment with Dr. MacDonald.

[110] With respect to whether she should have her right leg amputated below the knee, Ms. Bourgoin is faced with contradictory medical opinions. The municipality’s own experts do not recommend this surgery. Even Dr. Deathe said that she is not ready to have an amputation at this time for reasons that will be set out under Future Care.

[111] Is Ms. Bourgoin’s refusal of an amputation reasonable? It is my view that it is one thing to say that such an operation is objectively reasonable. It is quite a different thing to say that Ms. Bourgoin is acting unreasonably in refusing to have an amputation of part of her right leg. Refusing a back operation as the plaintiff did in *Janiak v. Ippolito* is not the same thing as refusing to have a major limb removed. It is my view that our law is not such that a refusal of an amputation can be considered unreasonable with the result that a plaintiff could be found to not have mitigated his or her losses.

[112] Whether a refusal to undergo a permanent mutilation of one’s body as treatment for chronic pain could ever be considered reasonable may be for another court on another day. On the facts of this case, and especially in the face of the opposing medical opinions, this is not such a case. I find that Ms. Bourgoin’s refusal to have her leg removed is not unreasonable. The

municipality has failed to meet its onus to establish that Ms. Bourgoin has failed to mitigate her loss.

Quantum of Non-Pecuniary Damages

Introduction

[113] In *Hossny v. Ramsoodar*, [2001] O.J. 5018, a case relied on by the defendant in support of its position on damages, the court said:

While it is well established that the quantum (of non-pecuniary damages) must be determined on the basis of the effect on the particular plaintiff and not simply by a comparison of awards made in other cases involving similar injuries, it is also recognized that the fairness of an award can be effected by the need to achieve the degree of consistency and uniformity, *Koukounakis v. Stainwood* (1995), 23 O.R. 3d 299 (C.A.) at p.p. 309-9 and cases cited there.

It is, therefore, necessary to consider Ms. Bourgoin's life before the accident and the effect that it has had on her life.

[114] Ms. Bourgoin was born one of 16 children. When her parents moved from Quebec to Leamington, Ms. Bourgoin was 14 years old and had only completed grade six. Nevertheless, her parents kept her out of school to look after the younger children while they worked in the fields. She went back to school as an adult and obtained her grade 10 equivalency. She also trained as a hairdresser. Three of her siblings died at birth and four others perished in a fire. Ms. Bourgoin met her husband on December 18, 1972. They were married on December 14, 1973 and had four daughters. In 1989, Mr. Bourgoin developed cancer. He had to have treatments, so the family moved back to New Brunswick so that he could be closer to his family. When they first arrived, the Bourgoin family was in receipt of public assistance. Ms. Bourgoin then opened her own business doing hairdressing and esthetics in order to support her family. The Bourgoin family moved back to Leamington in 1994.

[115] Ms. Bourgoin worked at a number of jobs including factory work, and as a general labourer planting tomatoes, weeding, picking peaches and corn.

[116] From 1983 to 1986, Ms. Bourgoin operated a "nursery" in her home. It was unclear in her evidence whether this was a licensed nursery or whether she babysat children in her home on an informal basis.

[117] From 1986 to 1987 Ms. Bourgoin worked in a nursing home where she cared for patients, gave medication and, at the same time, attended to housekeeping duties.

[118] On October 16, 1996, Ms. Bourgoin went to work at Highline Produce, a mushroom farm, where she was a "cutter" for six months, meaning that she sliced mushrooms.

She then became a “card marker” which meant that she was in quality control. She was classified as part-time but actually worked full-time hours. Her hours were generally from 6 a.m. to a maximum of 6 p.m. She was working full-time hours at the time of the fall.

[119] It is clear on the evidence that Ms. Bourgoin was not afraid of hard work and she faced challenges in her life by working hard and relying on her physical strength and ability to work hard as the basis for securing the necessities of life for herself and her family. There is nothing in her pre-fall life to suggest that Ms. Bourgoin would be inclined to avoid work by feigning illness or injury. In fact, as soon as she was cleared by her doctors to go back to work, she did so in June, 2001 on a reduced work schedule. After June, 2001, there is no evidence in the medical reports that Ms. Bourgoin ever requested that her doctors give her an order to stay off work.

[120] In spite of what many would think of as a hard life, Ms. Bourgoin was described by everyone as a happy and energetic person. Gustavo Gomez, her former supervisor at Highline, described her as a very good worker who got along with her co-workers and supervisors. She would help others, she respected everyone, was happy and seemed to enjoy her job. She was always smiling, and never raised her voice. This description of the plaintiff at work was confirmed by Lorenza Ortiz, a manager at Highline.

[121] Outside of work, Ms. Bourgoin loved to clean her house, work in her garden, spend time playing with her children and doing activities with them. She always had to be doing something and she especially loved to walk. She had a membership at a gym when she could afford it and, when she could not afford it, she worked out on equipment in their basement. In particular, she enjoyed spending time with her husband. She described her marriage as a good one. She would make her husband candlelight dinners and they would go out together to eat in restaurants. Her descriptions of her life and marriage were confirmed by the testimony of his sister, Ms. Colasanti, the plaintiff’s three daughters, and Paul Barkowski, Ms. Bourgoin’s neighbour.

[122] In addition to those interests, Ms. Bourgoin was very involved in her church. She was a minister of communion for 13 or 14 years until July 2000 when she fell. She had to stop for a while. She started again but had to quit because she could not carry out the duties on account of her injuries.

[123] Ms. Bourgoin said that prior to the accident, her goal had been to open her own beauty business. She said that she did not want to be rich but she wanted her family to be comfortable. She said that before the accident she loved her life.

[124] Ms. Bourgoin, by all accounts, including her own, became a different person after the fall. As the pain from her ankle spread to her leg and then to her back, she could not be as active as she had been and she became irritable, short-tempered both at home and at work. This once respected and liked employee was described by a manager shortly before her employment was terminated on January 7, 2003 as “high maintenance.” It appears from the medical evidence

that even some of the doctors found her behavior to be difficult. Dr. Soric found her to be “pain-focused” and dysfunctional, unwilling to help herself, exaggerating her symptoms and pain and not wanting to work. Although not a psychiatrist, Dr. Soric was of the view that Ms. Bourgoin needed psychiatric or psychological assistance because she thought the pain was not as bad as Ms. Bourgoin said it was. Dr. Pepin, who was her initial treating orthopedic surgeon, saw her a few times and then basically discharged her as a patient saying, “I am really at my wits end in terms of what I can do to help this patient, unfortunately, in my hands I am just not able to help Ms. Bourgoin and I advised her of the same.” Her never saw her again.

[125] Even as a witness at the trial, Ms. Bourgoin sometimes became very emotional. Nevertheless, it was my view that her pain was real. She was on the witness stand for two full days. Her extreme discomfort in not being able to stay in one position was evident as was the perspiration that would develop on her face as the day progressed. I found the following explanation by Dr. Deathe of Ms. Bourgoin’s emotional behavior helpful, compelling and compassionate:

Q. Over the years, in your practice of medicine, have you had the opportunity to observe patients who you felt were perhaps consciously exaggerating or magnifying their complaints or perhaps even outright malingering?

A. Well, the answer to that is “yes.” I see a lot of that. I see relatively few frank malingering; they’re usually picked up in surveillance tapes, I do a lot of defence work as well. That’s not as common as you’d think it is. What is more common is a person who has an injury and then runs into pain which is a stressful event, then over time runs into system problems with people believing him or her or not believing him or her and a lot of conflict, or conflict within the house, the husband or the wife not believing them, etcetera. And sometimes there’s a symptom magnification because after all, pain is invisible, it’s very hard to objectively analyze pain and some people are put in the situation where there’s symptom magnification and sometimes they’re doing it consciously and sometimes they’re doing unconsciously, but generally for a reason.

The problem is that unless you’ve had a lot of experience in this area, you tend to get put off by people that do that, but if you’ve had a lot of experience, I just take it as another complication, which to me is relatively natural; people will do that sometimes if they get frustrated or want to emphasize the situation to something that they can’t objectively prove otherwise. It doesn’t dissuade me, but it does dissuade some people.

He went on to say that he did not think Ms. Bourgoin was consciously magnifying her pain but was having difficulty coping with it.

Position of the Plaintiff

[126] Both counsel acknowledged that there was a great range in damages. Plaintiff's counsel urged me to find that, considering the effect on Ms. Bourgoin's life such as the loss of her jobs, the breakdown of her marriage and the extent of her pain and suffering that damages should be in the range of \$200,000. In taking that position, counsel for plaintiff relied on a number of cases.

[127] In *Alden v. Spooner* [2002] B.C.C.A. 592, the plaintiff was awarded \$200,000 for pain and suffering after being injured in four motor vehicle accidents in four years. She suffered from chronic pain and was depressed. The Court of Appeal dismissed the defendant's appeal and while acknowledging that the award was high, the injuries appeared to have had such a catastrophic effect on the plaintiff that the award was not so inordinately high as to be a wholly erroneous assessment of the loss. The defendant's further application for leave to appeal to the Supreme Court of Canada was dismissed with costs and without reasons on June 26, 2003.

[128] In another decision by the British Columbia Court of Appeal, *Soligo v. Turner* [2002] B.C.C.A. 73, the court dismissed the defendant's appeal and held that non-pecuniary damages of \$150,000 for non-pecuniary losses was not out of proportion to the injury and its consequences. In that case, however, the plaintiff had undergone seven surgeries, as well as numerous other treatments. It was also found that she lost her ability to follow her chosen career.

[129] In *Santi v. Pacific National Exhibition*, [2002] B.C.J. the court awarded \$150,000 in non-pecuniary damages to a woman who was injured when wooden room divider fell on her head. She was diagnosed with Regional Pain Syndrome.

[130] In *Teed v. Amero* [2001] N.S.J. No. 266, the Nova Scotia Supreme Court awarded a plaintiff \$150,000 on account of pain and suffering arising from a motor vehicle accident which left him disabled by excruciating pain which baffled the medical experts. The plaintiff, however, was confined to a wheelchair.

[131] In *Jaillet v. Allain* [1995] N.B.J. No. 314, the New Brunswick Court of Queen's Bench awarded an injured man \$150,000. He had soft tissue injury and a cervical spinal injury complicated by Reflex Sympathetic Dystrophy.

Position of Defendant

[132] It was the position of the defendant municipality that the plaintiffs in the cases relied on by Ms. Bourgoin were more severely injured and there was no possibility of medical treatment. The defendant was of the view that any damages awarded should be in the range of \$70,000 to \$90,000 prior to any deduction for the plaintiff's alleged failure to mitigate.

[133] The defendant relied on a number of cases in support of its position. In *D.O. v. Williams* [2001] O.J. No. 3594 the court assessed damages at \$125,000 in a case where the plaintiff had severe Reflex Sympathetic Dystrophy that had resulted in three amputations to her leg.

[134] In *Rowley v. Sobeys Inc.* [2000] N.B.J. No. 281, the New Brunswick Court of Queen's Bench assessed general damages at \$75,000 in a case where the plaintiff developed Reflex Sympathetic Dystrophy and was in constant pain.

[135] In *Nicoll v. Strata Plan 1611* [2005] B.C.J. No. 1177 the British Columbia awarded \$100,000 to a plaintiff who had a severe fracture resulting in two surgeries, skin grafts, little mobility in his ankle and a limp. He was also diagnosed with permanent and chronic pain that was not responsive to treatment.

[136] In *Hutchison v. Dalton* [2002] O.J. No. 4764 the Ontario Superior Court of Justice awarded \$50,000 to a plaintiff with a severe ankle injury.

[137] In *Hutchison v. Stratford-Perth Family Y.M.C.A.* [2004] O.J. No. 5751 a plaintiff who fell on a wet floor was awarded \$75,000 in general damages for chronic back pain caused by the fall.

Conclusion on Non-Pecuniary Damages

[138] In the case before me, Ms. Bourgoin's life has been permanently altered by the consequences of the municipality's negligence. She suffers chronic pain, has lost her ability to work and support herself as she has always been able to do because she was strong, healthy and willing to work hard. The chronic pain she experiences keeps her inactive, depressed and anxious about her future. Her marriage has ended, she is estranged from her oldest daughter and only beginning to restore her relationship with her youngest daughter. After applying the principles referred to in *Hossny v. Ramsooder, supra*, and having reviewed the authorities provided by counsel and taking into account the chronic and the debilitating pain that Ms. Bourgoin suffers and the effect her injuries have had on her life, I assess her non-pecuniary damages at \$125,000. There has been a significant disruption in her daily life. Her enjoyment of her family, of her work and of her social and recreational life represents a serious loss for her. I find that her current condition will likely continue.

5) SPECIAL DAMAGES FOR PAST CARE

[139] The parties agree that Ms. Bourgoin incurred expenses for care she received after the fall. The only disagreement relates what expense should be allowed on account of the cost of medication. There was no evidence presented that Green Shield has actually made a subrogated claim or, indeed if it has the right to make such a claim. I, therefore, fix the cost of Ms. Bourgoin's medication to date at \$553.52, the actual amount she has spent out-of-pocket which, according to the document at Tab 4 of the expense brief, is \$553.52.

[140] In addition to medication costs of \$ 553.52,

I have allowed

cost of cab on July 6, 2000 \$ 5.00

ankle brace \$ 64.95

ankle weight \$ 9.16

walking brace \$ 270.00

theraband \$ 7.50

transportation \$ 238.50

OHIP \$ 5,313.65

notes from Dr. Gorrell \$ 34.00

Total \$ 6,496.28

[141] The special damages for past care, therefore, amount to \$6,496.28. If I have made a mathematical error or omitted an item, I may be spoken to by counsel and I will amend the amount awarded.

6) SPECIAL DAMAGES FOR PAST WAGE LOSS

[142] The parties agree that Ms. Bourgoin suffered a loss of income due to the accident from July 7, 2000 to June 4, 2001 when she was totally off work and from June 4, 2001 to January 7, 2003 on account of the reduction in the hours she was able to work because of her injuries. The main issue in contention is whether from January 7, 2003 to trial she failed to mitigate this loss by not accepting a lighter duty job that the municipality alleges she was offered and refused.

[143] The basis for the municipality's position is the circumstances surrounding the termination of Ms. Bourgoin's employment from Highline. Prior to the termination letter, management met with her about the difficulties she had fulfilling her job responsibilities because of her increasing physical limitations. In the letter dated January 7, 2003 detailing the problems submitted as part of Exhibit 4, the Human Resources Director indicated that Ms. Bourgoin had

been offered to try a position in the packing room. She had been given the physical demands analysis for the jobs to review and it appears that she decided she could not do the job. Having made that decision, it appears that the employer was unwilling to accept her change of mind about the other jobs that she expressed on January 7, 2003.

[144] Based on the physical demands analysis for the packing room job and the evidence of Mr. Ortiz, it is my view that Ms. Bourgoin should have tried to do the job that was suggested to her. On the other hand, based on the medical reports that have been made exhibits by both other parties from January 2003 and onward and the fact that she started taking duragesic medication by May 2003, it is more likely than not that Ms. Bourgoin would not have been able to continue to work until trial. I find that she could not have worked longer than a year.

[145] In addition to the medical evidence, I also based this finding on the evidence provided by Mr. Gomez and Mr. Ortiz about the unsatisfactory performance of Ms. Bourgoin after the accident which included changes in her personality. As well, the e-mail of May 2002 between Highline Manager, Art Williams and Human Resources Manager, Lynn Nettleton discussed the employer's concern regarding Ms. Bourgoin's behavior with other people since her return to work and the fact that she was "high maintenance." The totality of the evidence about her problems at work, combined with the deterioration in her medical condition leads me to find that she failed to mitigate her loss by not accepting the lighter position but that this failure is only for a 12 month period to January 7, 2004.

[146] Therefore, I find that Ms. Bourgoin is entitled to special damages for past wage loss for the period from the day after the fall, July 7, 2000 to June 3, 2001 when she returned to work three days per week. From June 3, 2001 to January 7, 2004, her loss is calculated at two days per week. From January 2004 until trial, her loss is five days per week. Based on the wage information provided by the plaintiff, the quantum should be able to be agreed on by counsel. If not, counsel may make submissions to me arranged through the trial coordinator.

7) FUTURE INCOME LOSS

[147] The next issue is whether Ms. Bourgoin is totally disabled from working either on a full-time or part-time basis. If I find she is not totally disabled, I must decide when she might be able to return to work, what kind of work that could be and whether she could return to work on a full-time or part-time basis.

[148] The municipality urged me to find that Ms. Bourgoin is not totally disabled. In taking that position it relied on the evidence of Dr. Ansell and Dr. Deathe's evidence that an amputation has a 95% chance of relieving her pain and that after a period of rehabilitation, she could return to work. They also relied on the evidence of Dr. Soric that she could return to work right now, on a part-time basis to begin with.

[149] In *Hutchinson v. Stratford-Pirth Family Y.M.C.A.*, *supra*, the court considered the standard of proof of a future loss of income. It relied on the Ontario Court of Appeal decision in *Schrump v. Koot* (1977), 18 O.R. (2d) 337 which held:

It is not necessary for him (Plaintiff) to prove on the balance of probabilities that future loss or damage will occur but only that there is a reasonable chance of such loss or damage occurring.

[150] Based on the evidence of Ms. Bourgoin's medical condition and the effects of the medication as well as her limited education, I find that she is not likely to be able to work either full or part-time in the future. If she were to have an amputation within the next five years, Dr. Deathe said it would take at least a year of rehabilitation. Even if she adjusted to wearing prosthesis, and her pain was under control, her chances of getting employment would have to take into account, as pointed out by Dr. Ansell, that she would be competing for unskilled jobs with younger, able-bodied prospective employees. By that time Ms. Bourgoin will be 57 years old. Her loss of income will be calculated to age 65 when other benefits are available to her.

8) FUTURE CARE COSTS

[151] The final issue to be determined in phase one of the trial is whether Ms. Bourgoin will require future care and whether that care may include a below the knee amputation.

[152] It was the position of the municipality that unless Ms. Bourgoin undergoes an amputation at some time in the next five years, there is little, if any, future care required by her except for medication costs in the immediate future. The municipality argued that, in fact, according to Dr. Soric, any future care would be counter-productive as it would encourage Ms. Bourgoin to be less mobile and mobility is important. The municipality's position was that most doctors thought that Ms. Bourgoin had received all of the available treatment.

[153] Other than the possibility of an amputation which I will deal with below, there is evidence on which I find that she does, in fact, require future care and will be entitled to the costs of that care. Dr. Deathe and Dr. Gorrell as well as Dr. Merskey said that Ms. Bourgoin will require prescription medications to deal with the pain. If she is not able to maintain her prescription benefits because of her separation from her husband, she will require the actual costs of the medication.

[154] Both Dr. Deathe and Dr. Merskey thought a consultation at the Cleveland Clinic or the Mayo Clinic could be of assistance with respect to other treatments available.

[155] Most of the housekeeping and gardening responsibilities are now undertaken by Ms. Bourgoin's daughter, Natalie. The plaintiff is no longer able to do the work because of her pain. Natalie, however, has applied to join the Navy and is optimistic about her chances of acceptance in the near future. This will mean that Ms. Bourgoin will require some housekeeping help, perhaps up to eight hours per week. Based on the evidence, it would make sense if Ms. Bourgoin moved into an apartment, thus eliminating the need for gardening assistance.

[156] Dr. MacDonald has recommended that Ms. Bourgoïn receive psychotherapy to help her cope with the pain which will hopefully then reduce her depression and anxiety. He estimated a minimum of 50 sessions at \$150 to \$200 per hour.

[157] Regarding the possibility of an amputation, I find that the evidence does not support a finding that it is more likely than not that Ms. Bourgoïn will have one. Although Dr. Deathe said that it is an effective treatment for intractable pain, he acknowledged that Ms. Bourgoïn was not ready to have this surgery. Dr. Deathe also said that an amputation is “not a strategy I would recommend right now until she’s – we’ve had a neuropathic pain physician look at her such as Dwight Moulin or at the Cleveland Clinic and if there is further pharmacologic strategies At the present time she may or may not fit into that – that cohort of cases that would be successful. We haven’t had a psychologist or psychiatric opinion of her and she’s not ready for it ... and I’m not saying if she agreed tomorrow that we would do it. I’m saying that if she agreed tomorrow, we would put her through the process of evaluation.” He was of the view that with other professionals on board with respect to the amputation it would assist to “more authoritatively persuade the surgeon that in this case, an amputation is reasonable. So it is for my benefit as well as her benefit.”

[158] Dr. Deathe went on to describe that the people who needed to be on board in addition to himself in order to proceed with an amputation would be: the patient, the family doctor, a psychologist or psychiatrist and a surgeon. Dr. Deathe said “If there’s conflict, you wouldn’t get a surgeon within 100 miles of her.” Dr. Yovanovich, the only orthopaedic surgeon who testified echoed that sentiment. When asked about the advisability of an amputation for Ms. Bourgoïn, he replied that he would not do it because he valued his licence. There is no other contradictory opinion. Dr. Gorrell said that every other treatment should be considered before considering an amputation.

[159] On the totality of the medical evidence, I find that if Ms. Bourgoïn has an amputation, it will not be in the near future. Dr. Deathe said it would likely happen within the next five years. If it does happen, I find on the evidence that it will be later rather than sooner. The longer she goes without it and if Dr. MacDonald is successful in assisting her to cope with the pain, I find that the likelihood of amputation will be reduced. I have reduced the likelihood of an amputation by 20% for each of the next five years that pass without her having the amputation. I would, therefore, only assign a 20% chance to the likelihood of an amputation.

SUMMARY

[160] In conclusion, the following are the answers to the questions posed by counsel at the commencement of the trial:

Liability

1. **Was the Town of Leamington negligent with respect to maintaining the sidewalk in question?**

Yes.

2. If yes, then how?

By failing to maintain the sidewalk in a reasonable state of repair and by violating their own repair policy of patching holes over ½” deep.

3. Did the negligence of the Town of Leamington cause or contribute to the fall?

Yes.

Contributory Negligence

4. Was Martine Bourgoin contributorily negligent?

No. The municipality failed to establish on the evidence that Ms. Bourgoin contributed to the fall.

5. If yes, then how?

N/A.

6. To what extent expressed as percentage?

N/A.

Causation

7. Was Martine Bourgoin injured as a result of the fall?

Yes.

8. If yes, what is the nature and extent of the injuries suffered?

The injured ankle and its failure to heal or improve resulted in Ms. Bourgoin developing a chronic intractable pain condition which Dr. Deathe diagnosed as Complex Regional Pain Syndrome, a diagnosis I accepted, based on the totality of the medical evidence.

Non-pecuniary General Damages

9. Has Martine Bourgoin failed to mitigate her damages under this heading?

No. Her refusal to have a below the knee amputation is not unreasonable.

10. What amount should be awarded under this heading?

\$125,000 on account of pain and suffering, taking into account the extent of her injuries and the effect the injuries have had on her life and will have in the future.

Special Damages for Past Care

11. What amount should be awarded under this heading?

\$6,496.28. This amount excluded any cost of medication over and above the actual cost to Ms. Bourgoin.

Special Damages for Past Wage Loss

12. Has Martine Bourgoin suffered a loss of income due to her accident related injuries since July 6, 2000 to the date of judgment?

Yes.

13. If yes, for which period or periods?

July 7, 2000 to June 4, 2001 – full-time, January 7, 2003 to January 7, 2004 – 2 days per week loss, January 7, 2004 to trial – full-time.

14. Did Martine Bourgoin fail to mitigate her losses during the said period or periods?

Yes.

15. If yes, how and for which period(s) of time?

January 7, 2003 to January 7, 2004.

Future Loss of Income

16. Is Martine Bourgoin totally disabled with respect to any meaningful gainful employment on either a full or part-time basis?

Yes.

17. If not, when is it expected that she will return to work, in what capacity and on what terms?

N/A.

- 18. Has Martine Bourgoin suffered a loss of shared family income? (not agreed to by the defendant and to be argued in closing)**

Withdrawn by plaintiff.

Future Care

- 19. Will Martine Bourgoin require future care?**

Yes.

- 20. Will Martine Bourgoin, more likely than not, require a below the knee amputation?**

No.

- 21. If yes, when?**

N/A.

- 22. If no, is it a possibility?**

Yes.

- 23. If yes, what is that possibility expressed as a percentage?**

20%.

If any of my reasons need clarification, I may be spoken to by counsel at a time to be arranged with the trial coordinator. Phase II of the trial may also be scheduled with the trial coordinator. I am grateful to counsel for Ms. Bourgoin and the municipality for their assistance in a case that raised difficult and unusual issues.

"original stamped Justice Nolan"

Mary Jo M. Nolan
Justice

Released: April 13, 2006

COURT FILE NO.: 00-GD-49880

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

MARTINE BOURGOIN

Plaintiff

- and -

THE CORPORATION OF THE MUNICIPALITY
OF LEAMINGTON

Defendant

REASONS FOR JUDGMENT

Nolan J.

Released: April 13, 2006

